

22STCV02241 22STCV02241

County: los-angeles

Division: Civil Law and Motion

Department: C

Hearing date: 2026-09-08

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Case Number: 22STCV02241 Hearing Date: September 8, 2026 Dept: C

GONZALES v. PIH HEALTH HOSPITAL – WHITTIER, et al.

CASE NO.:

22STCV02241

HEARING: 9/8/26

@ 9:30 AM

#11

TENTATIVE

ORDER

Defendant BARLOW RESPIRATORY HOSPITAL's Motion for Summary
Adjudication of the second cause of action for Elder Abuse is DENIED.

Plaintiff to give Notice.

Defendant BARLOW RESPIRATORY HOSPITAL

(Defendant or Barlow) moves for summary adjudication of Plaintiff WANDA M.
GONZALES's (Plaintiff) second cause of action for Elder Abuse as alleged in
Plaintiff's Third Amended Complaint.

Plaintiff, the surviving spouse of Defendant's patient and decedent DANIEL
GONZALES (Decedent), sues individually and as the personal representative of
the ESTATE of DANIEL GONZALES.

On February 5, 2026, this Court denied BARLOW's

Motion for Summary Judgment. Additionally, BARLOW's Motion for Summary

Adjudication of the first cause of action for Professional Negligence (Medical Malpractice) was DENIED as to standard of care and GRANTED as to causation.

On April 2, 2026, the Court granted Defendant
PIH HEALTH WHITTIER – HOSPITAL's Motion for Summary Judgment.

BACKGROUND

This is a medical malpractice case. The operative Third Amended Complaint (TAC) alleges that Decedent was admitted to PIH – WHITTIER (PIH) as an inpatient on or about June 8, 2020 for various medical issues that arose as a result of his eventual diagnosis of COVID-19, including but not limited to pneumonia due to severe acute respiratory syndrome corona-virus; acute respiratory failure; adult respiratory distress syndrome; bacterial pneumonia; deep venous thrombosis, and GI bleeding. Decedent was placed under the care of PIH and its employees. (TAC, ¶ 16.)

On or about July 20, 2020, Decedent was discharged and transferred from PIH to Barlow. Upon admission to Barlow, he was diagnosed with a Stage III sacral decubitus ulcer wound ("Pressure Ulcer"). At no point during his residency at PIH was a bed sore diagnosed, identified, addressed or treated. In fact, PIH repeatedly documented Decedent's skin as dry and intact. However, at some point during his stay at PIH, a bedsore had formed and had gone undetected and ignored to the point where it ultimately developed to Stage III by the time Decedent was discharged. (TAC, ¶ 17.)

During his stay at Barlow, from July 20, 2020 until his discharge on March 23, 2021, the Pressure Ulcer advanced to Stage IV. (TAC, ¶ 22.)

In August, 2021, and again in October of 2021, Barlow failed to properly and adequately monitor Decedent's blood oxygen levels which led to instances of severe hypoxemia and a collapsed lung. (TAC, ¶ 28.)

Once discharged from Barlow, PIH provided Decedent with home care services from March 24, 2021 until February 2022. (TAC, ¶ 29.) On or about February 18, 2022, Decedent passed away. (TAC, ¶ 30.)

Plaintiff alleges Decedent's injuries and ultimate passing occurred due to Defendants PIH and Barlow's failure to exercise reasonable care. Based thereon, the operative Third Amended Complaint

asserts causes of action for: 1) Professional Negligence (Medical Malpractice); 2) Violation of Elder and Dependent Adult Civil Protection Act; and 3) Wrongful Death.

EVIDENTIARY

OBJECTIONS

In granting or denying a motion for summary judgment or summary adjudication, the court need rule only on those objections to evidence that it deems material to its disposition of the motion. Objections to evidence that are not ruled on for purposes of the motion shall be preserved for appellate review. (Code Civ. Proc., § 437c, subd. q.)

Plaintiff's

Objection Nos. 1-10 to the declaration of Oliver D. Tomas are OVERRULED.

Barlow's

Objection Nos. 1, 2, 3, 4, 6-14, 15-20 are OVERRULED.

LEGAL

STANDARD

A party seeking summary judgment has the burden of producing evidentiary facts sufficient to entitle them to judgment as a matter of law. (CCP § 437c(c).) The moving party must make an affirmative showing that they are entitled to judgment irrespective of whether or not the opposing party files an opposition. (Villa v. McFerren (1995) 35 Cal.App.4th 733, 742743.) Thus, "the initial burden is always on the moving party to make a prima facie showing that there are no triable issues of material fact." (Scalf v. D. B. Log Homes, Inc. (2005) 128 Cal.App.4th 1510, 1519 (citing Aguilar v. Atlantic Richfield Co. (2001) 25 Cal.4th 826, 850.)

When a plaintiff seeks summary judgment, they must produce admissible evidence on each element of each cause of action on which judgment is sought. (CCP § 437c(p)(1).) When a defendant seeks summary judgment, they have the "burden of showing that a cause of action has no merit if the party has shown that one or more elements of the cause of action, even if not separately pleaded, cannot be established, or

that there is a complete defense to the cause of action.” (CCP § 437c(p)(2).)

The opposing party on a motion for summary judgment is under no evidentiary burden to produce rebuttal evidence until the moving party meets their initial movant’s burden. (*Binder v. Aetna Life Insurance Company* (1999) 75 Cal.App.4th 832, 840.) Once the initial movant’s burden is met, then the burden shifts to the opposing party to show, with admissible evidence, that there is a triable issue requiring the weighing procedures of trial. (CCP § 437c(p).) The opposing party may not simply rely on their allegations to show a triable issue but must present evidentiary facts that are substantial in nature and rise beyond mere speculation. (*Sangster v. Paetkau* (1998) 68 Cal.App.4th 151, 162.) Summary judgment must be granted “if all the evidence submitted, and ‘all inferences reasonably deducible from the evidence’ and uncontradicted by other inferences or evidence, show that there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law.” (*Adler v. Manor Healthcare Corp.* (1992) 7 Cal.App.4th 1110, 1119.)

DISCUSSION

Barlow moves for summary adjudication on the grounds that the hospital services provided by Barlow to Plaintiff do not implicate elder abuse or neglect because the facts do not present issues of “custodial caretaking,” a prerequisite for maintaining such a cause of action. (*Winn v. Pioneer Medical Group, Inc.* (2016) 63 Cal.4th 148, 152.)

Elder Abuse

The Elder Abuse Act defines abuse as “[p]hysical abuse, neglect, financial abuse, abandonment, isolation, abduction, or other treatment with resulting physical harm or pain or mental suffering” (*Welf. & Inst.Code*, § 15610.07, subd. (a), *italics added*); or “[t]he deprivation by a care custodian of goods or services that are necessary to avoid physical harm or mental suffering” (*id.*, § 15610.07, subd. (b)). The Act defines neglect as “[t]he negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise.” (*Id.*, § 15610.57, subd. (a)(1).)

[S]everal

factors [] must be present for conduct to constitute neglect within the meaning of the Elder Abuse Act and thereby trigger the enhanced remedies available under the Act. The plaintiff must allege (and ultimately prove by clear and convincing evidence) facts establishing that the defendant: (1) had responsibility for meeting the basic needs of the elder or dependent adult, such as nutrition, hydration, hygiene or medical care; (2) knew of conditions that made the elder or dependent adult unable to provide for his or her own basic needs; and (3) denied or withheld goods or services necessary to meet the elder or dependent adult's basic needs, either with knowledge that injury was substantially certain to befall the elder or dependent adult (if the plaintiff alleges oppression, fraud or malice) or with conscious disregard of the high probability of such injury (if the plaintiff alleges recklessness). The plaintiff must also allege (and ultimately prove by clear and convincing evidence) that the neglect caused the elder or dependent adult to suffer physical harm, pain or mental suffering. Finally, the facts constituting the neglect and establishing the causal link between the neglect and the injury "must be pleaded with particularity," in accordance with the pleading rules governing statutory claims. (Carter v. Prime Healthcare Paradise Valley LLC (2011) 198 Cal.App.4th 396, 406-407, citations omitted.)

Acts

which are prohibited under the Elder Abuse Act provisions of the Welfare and Institute Code "do not include acts of simple professional negligence, but refer to forms of abuse or neglect performed with some state of culpability greater than mere negligence." (Covenant Care, Inc. v. Superior Court (2004) 32 Cal.4th 771, 781 (Covenant Care).) To constitute abuse and neglect within the meaning of the Elder Abuse Act, thereby triggering the enhanced remedies available under the Act, a plaintiff "must demonstrate by clear and convincing evidence that defendant is guilty of something more than negligence; he or she must show reckless, oppressive, fraudulent, or malicious conduct," with recklessness referring "to a subjective state of culpability greater than simple negligence, which has been described as a 'deliberate disregard' of the 'high degree of probability' that an injury will occur," and '[o]ppression, fraud and malice [involving] intentional or conscious wrongdoing of a despicable or injurious nature.'" (Sababin v. Superior Court (2006) 144 Cal.App.4th 81, 88-89, quotation marks omitted.)

"[N]eglect

refers not to the substandard performance of medical services but, rather, to the failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations.' Thus, the statutory definition of 'neglect' speaks not of the undertaking of medical services, but of the failure to provide medical care." (Covenant Care, Inc. v. Superior Court (2004) 32 Cal.4th 771, 783.)

Barlow argues that the provision of hospital-medical services is not custodial care because such services are not things able-bodied, competent persons would do for themselves. (Winn v. Pioneer Medical Group, Inc. (2016) 63 Cal.4th 148.) Barlow analogizes to Oroville Hospital v. Superior Court (2022) 74 Cal.App.5th 382, where the Court explained that although a hospital's home health agency could be a care custodian under the statutory definition that the issues did not implicate elder-dependent abuse. (Id. at 388.)

In support, Barlow relies on the evidence that the facts alleged in the Third Amended Complaint demonstrate that Barlow provided professional medical services to Decedent, not basic custodial caretaking services that anyone would ordinarily be able to provide for themselves. Barlow's staff made medical decisions for a patient of advanced age who arrived at the hospital suffering from acute respiratory failure and serious comorbidities.

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While admitted to Barlow, Mr. Gonzales continued to receive treatment primarily related to the complications associated with his COVID-19 conditions. The major focus of his treatment was the respiratory issues. (Exh. CC, Third Amended Complaint, ¶ 24.)

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Plaintiff states that Barlow "failed to properly and adequately monitor his blood oxygen levels which led to instances of severe hypoxemia and collapsed lung. (Exh. CC, Third Amended Complaint, ¶ 27.)

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Further, the Third Amended Complaint states:

"While [Mr. Gonzales] was a patient at Barlow, Barlow failed to properly care

for his bedsores such that the Stage III Pressure Ulcer documented when it first began to care for him advanced to a Stage IV Pressure Ulcer.” (Exh. CC, Third Amended Complaint, ¶ 25.)

The Court finds that Barlow has not met its initial burden of showing that no triable issue of material fact exists regarding whether Barlow had a custodial relationship vis a vis Decedent. The Court finds both Winn and Oroville Hospital distinguishable. In both cases defendants were relied upon by the decedent for intermittent, outpatient care. In Winn, defendants treated the decedent at outpatient clinics operated by defendants, plaintiffs offered no other explanation for why defendants' intermittent, outpatient medical treatment forged a caretaking or custodial relationship between the decedent and defendants. The allegations in the case failed to support an inference that the decedent relied on defendants in any way distinct from an able-bodied and fully competent adult's reliance on the advice and care of his or her medical providers. (Winn, *supra*, 63 Cal.4th at p. 160.)

Similarly, in Oroville Hospital, the Court reasoned that Defendants' provision of in-home nursing for wound care on a number of occasions is more analogous to the outpatient visits the decedent had with physicians in Winn than it is the situation where a defendant has “assumed significant responsibility for attending to one or more of those basic needs of the elder or dependent adult that an able-bodied and fully competent adult would ordinarily be capable of managing without assistance.” (Oroville Hospital v. Superior Court (2022) 74 Cal.App.5th 382, 406.)

Here, by contrast, Decedent was an inpatient at Barlow, from July 20, 2020 until his discharge on March 23, 2021. (TAC, ¶ 22.) Upon

admitting Decedent as an inpatient, Barlow undertook the “responsibility for meeting the basic needs of the elder [], such as nutrition, hydration, hygiene or medical care,” as required for a custodial relationship by the statute. (Carter v. Prime

Healthcare Paradise Valley LLC (2011) 198 Cal.App.4th 396, 406-407.) Moreover, the allegations state that upon admission Decedent had

impaired mobility. (TAC, ¶ 20.) Plaintiff

alleges that Decedent required “custodial care” in addition to “appropriate medical monitoring therapy, healthcare and nutrition, hygiene, and all that one would expect of an acute care facility. (TAC, ¶ 31.) Unlike the decedents in Winn and Oroville Hospital, as an inpatient Decedent relied on Barlow for both the provision of basic needs and medical care. Thus, Barlow fails to demonstrate no triable issues of material fact exist as to whether there was a custodial relationship between

Barlow and Decedent.

Causation

As to causation, Barlow argues that “[i]n the present case, the declarations of Dr. Leo and Ms. Higelin establish that no act or omission by Barlow caused the patient’s physical injury, pain, loss of enjoyment of life and emotional distress and anguish ... As such, plaintiff is unable to establish that a probable causal connection exists between any alleged act and/or omission by Barlow and the patient’s physical injury, pain, loss of enjoyment of life and emotional distress and anguish.” (Motion, p. 15: 5-6.)

Barlow submits the following evidence in support:

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It is my opinion that the injuries alleged in this case are not the result of the care and treatment the patient received while he was a patient at Barlow. Beginning with the pressure wound on the patient’s coccyx, according to the wound management records and photographs, the area of the wound decreased by approximately 50%, and at the time of discharge from Barlow demonstrated substantial granulation tissue in the wound bed. Exhibit J - Barlow Respiratory Hospital, pp. 1665- 1668; Exhibit K - Barlow Respiratory Hospital, pp. 1614-1617; Exhibit L - Barlow Respiratory Hospital, pp. 1669-1819. The development of the pressure wound could not be attributed to Barlow since the wound clearly originated prior to the patient’s admission there. (Exhibit Y – Leo Decl., ¶ 22.)

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Regarding the risk factors associated with the development and progression of pressure wounds in general, the patient’s risk factors included diabetes, administration of corticosteroids, and significantly low albumin due to a chronic catabolic state. Exhibit Q - Barlow Respiratory Hospital, pp. 653-655; Exhibit R - Barlow Respiratory Hospital, pp. 970-971. Diabetes can impair wound healing through mechanisms of macrovascular and/or microvascular disease impeding oxygen delivery to tissues, and due to the adverse metabolic impact of hyperglycemia on wound healing. Corticosteroids cause an enzymatic switch in from anabolism (the ability to build protein) to catabolism (a state in which proteins that comprise muscle and connective

tissue are broken down). (Exhibit Y – Leo Decl., ¶ 23.)

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The patient also had a significantly low albumin level that hovered around 2.0 (4.0 is normal) due to his chronic catabolic state, and despite appropriate nutritional therapy being provided. Exhibit Q - Barlow Respiratory Hospital, pp. 653-655. The chronic catabolic state was a result of the effects of COVID and post-COVID chronic lung disease, followed by serial infections due to the COVID-induced lung damage and having a tracheostomy. The presence of a tracheostomy tube bypasses the normal protective mechanisms of the upper airway, increasing the risk of pneumonia. When the body is in a catabolic state, the body's ability to synthesize albumin is impaired despite proper nutrition. Albumin is the key protein responsible for maintaining tissue integrity, given its key role in preventing tissue edema and its function as a transport protein carrying molecules necessary for tissue support and healing. The very low albumin in this case served as an additional non-preventable factor impairing the ability of Mr. Gonzalez's sacral wound to heal, despite appropriate wound care. (Exhibit Y – Leo Decl., ¶ 24.)

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The patient's severe post-COVID condition prolonged the inflammatory state as evidenced by a chronically elevated C-reactive protein level (a key marker of inflammation). This prolonged inflammatory state also impaired wound healing in this patient. (Exhibit Y – Leo Decl., ¶ 25.)

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Surface wound cultures were positive for bacteria. However, a chronic wound always has bacteria on its surface, and surface wound cultures have no predictive value regarding the presence of invasive wound infection, nor of the specific infecting organisms if deep tissue infection is present. This could only be determined with tissue biopsy and/or deep wound fluid aspiration and culture. However, the partial healing of the wound as demonstrated by a generally progressive reduction in wound area throughout the patient's admission is suggestive that no deep tissue infection was present, and at no time did Dr. Doo, the infectious disease specialist, diagnose Mr. Gonzalez as having infection of or related to the sacral pressure ulcer. Exhibit M - Barlow Respiratory Hospital, pp. 77-634. 27. (Exhibit Y – Leo Decl., ¶ 26.)

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The patient also appeared to suffer from critical illness myopathy, a condition in which denervation from critical illness-related inflammation (commonly, as in this case, due to sepsis) leads to muscle weakness and atrophy. Exhibit S - Barlow Respiratory Hospital, pp. 184-185. The resulting loss of muscle mass means a loss of the cushion between the skin and bone in the area of pressure points, including the coccyx region. This results in an increased risk of developing pressure wounds and impaired healing for pressure wounds that have already developed. (Exhibit Y – Leo Decl., ¶ 27.)

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With regards to the alleged episodes of desaturation of the patient, an arterial blood gas obtained on August 30, 2020, reflected the patient to be in a state of acute-on-chronic respiratory acidosis consistent with the effects of the COVID-19 infection and its resultant pulmonary damage. At 05:30 hours on August 30, 2020, Mr. Gonzalez's nurse assessed him and found that he had labored breathing and decreased level of consciousness. His arterial blood gas showed a pO₂ of 67 with an O₂ saturation of 96.2%. Mr. Gonzalez then desaturated to 75% and was placed back on the ventilator, with a rise in his pO₂ to 264 and O₂ saturation of 99.9%. Exhibit T - Barlow Respiratory Hospital, pp. 172- 173; pp. 1500-1501. On August 29, Mr. Gonzalez had been noted by his pulmonary consultant, Dr. Dharmi Patel to have right lower lobe atelectasis and left lower lobe consolidation, despite which the patient had been tolerating being off the ventilator and on a tracheostomy collar for over two weeks. Exhibit U - Barlow Respiratory Hospital, pp. 168-169. The occurrence of the desaturation on August 30, 2020 was not foreseeable. It was managed appropriately by the nursing and respiratory therapy staff, as directed by the on-call physician. There is no evidence that this episode produced any lasting harm to Mr. Gonzalez. This episode was a manifestation of Mr. Gonzalez's severe post-COVID pulmonary disease, and was not the cause of any subsequent problems. (Exhibit Y – Leo Decl., ¶ 28.)

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The patient also suffered atelectasis (a collapsed portion of the lung) due to mucus plugging. Exhibit T - Barlow Respiratory Hospital, pp. 172-173. This condition can and does occur even in the best of care in a situation such as this due to the excess mucus production caused by the COVID-induced lung damage. There are multiple modalities utilized to treat atelectasis, including frequent suctioning, inhalational therapy,

chest physiotherapy, positive pressure ventilation, and bronchoscopy. Here, the treating pulmonologist, Dr. Patel, in fact considered bronchoscopy for the patient, which was eventually performed on September 3, 2020. Exhibit T - Barlow Respiratory Hospital, pp. 172-173. (Exhibit Y – Leo Decl., ¶ 29.)

The only other desaturation event seen in the records occurred on October 6, 2020, when the patient's O2 saturation fell transiently to 88% after an episode of vomiting. Responding staff increased the inspired oxygen concentration to 100%, suctioned the patient, and he was placed back on full ventilator support. Exhibit W - Barlow Respiratory Hospital, pp. 278-279. The occurrence of vomiting followed by this minor degree of desaturation was non-foreseeable and non-preventable, and was responded to promptly and appropriately. (Exhibit Y – Leo Decl., ¶ 31.)

Based on the foregoing, it is my professional opinion, to a reasonable degree of medical probability, that no act or omission by Barlow caused or contributed to physical injury, pain, loss of enjoyment of life and other forms of severe mental and emotional distress and anguish of the patient. The physical injury, pain, loss of enjoyment of life and other forms of severe mental and emotional distress and anguish of the patient was a direct consequence of his original COVID-19 infection and was not the result of anything the healthcare providers at Barlow did or failed to do in this case. (Exhibit Y – Leo Decl., ¶ 33.)

Finally, the patient's underlying comorbidities were primary barriers to healing the wound completely. Specifically, the patient had persistently low albumin, which impairs wound healing. Other factors included diabetes, administration of corticosteroids, and pneumonia. Exhibit Q - Barlow Respiratory Hospital, pp. 653-655; Exhibit R - Barlow Respiratory Hospital, pp. 970-971. These factors impair wound healing by diminishing perfusion of blood to the extremities and skin. The body's natural reaction to pneumonia is to preserve major organ function, which results in resources, including blood flow, being diverted from the skin to the major organs and to fight infection. Moreover, the patient died nearly a year after he was discharged home from Barlow, and the cause of death was identified as respiratory failure due to complications from COVID-19. Exhibit P – Daniel Gonzalez's death certificate. (Exhibit Z – Higelin Decl., ¶ 26.)

The Court finds that Barlow has met its initial movant's burden on the issue of causation as presented in the moving papers. As the Court previously noted, Barlow's expert opines that as to respiratory related care, Decedent's collapsed lung can and does occur even in the best of care in a situation such as this due to the excess mucus production caused by the COVID-induced lung damage. Treatment may include atelectasis, including frequent suctioning, inhalational therapy, chest physiotherapy, positive pressure ventilation, and bronchoscopy. Here, the treating pulmonologist, Dr. Patel, in fact considered bronchoscopy for the patient, which was eventually performed on September 3, 2020. Moreover, Dr. Leo declares that the only other desaturation event seen in the records occurred on October 6, 2020, when the patient's O2 saturation fell transiently to 88% after an episode of vomiting. Responding staff increased the inspired oxygen concentration to 100%, suctioned the patient, and he was placed back on full ventilator support. (Exhibit W - Barlow Respiratory Hospital, pp. 278-279.) Although, as stated above, Plaintiff alleges that the improper monitoring of Decedent's blood oxygen levels occurred in or about August, 2021, and again in October of 2021, the record provides that the only two "desaturation" events occurred in or about August 2020 and October 2020. (Exhibit W - Barlow Respiratory Hospital, pp. 278-279.) Therefore, the Court finds that Defendant has made a prima facie showing that a triable issue of material fact does not exist.

The burden shifts to Plaintiff to raise a triable issue of material fact. Plaintiff argues that Barlow's no-injury argument depends on a selective narrative: the wound began as Stage III, its surface area became smaller, and therefore it improved. However, Plaintiff contends that there are contrary measurements of Decedent's wound and the existence of a Stage IV treating-physician entries further demonstrate the discrepancy in Decedent's medical record while an inpatient at Barlow. Additionally, Plaintiff's expert opines that "superficial shrinkage accompanied by major deep-tissue loss was not healing but undermining and progression." (Katona Decl., paras. 4(e), 11-12.)

As to causation, Plaintiff submits the following evidence:

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Deborah Collier appeared as Barlow's

designated person most qualified, testified on behalf of Barlow, and understood her testimony was Barlow's testimony. (Collier Depo. 6:16-24.)

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Barlow's records also identify the wound as Stage III and Stage IV on March 17, 2021; Collier confirmed both were authentic Barlow records. (Collier Depo. 73:21-74:4.)

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Collier could not determine who created the conflicting March 17 entries, whether either was carried forward, or whether the person entering Stage III reviewed Dr. Doo's Stage IV record. (Collier Depo. 74:5-18.)

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Dr. Katona reviewed the Dakin's and Santyl treatment and nevertheless opined that the depth increase reflected deep-tissue loss and progression rather than healing. (Katona Decl. paras. 4(e), 11-12.)

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The Court's prior causation adjudication was expressly limited to damages attributable to Daniel's ultimate death and did not preclude damages unrelated to death.

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The TAC alleges Barlow was a long-term, twenty-four-hour health facility and care custodian; Daniel's Stage III wound advanced to Stage IV; Barlow failed to develop and implement care plans for his nursing, nutritional, and pressure-ulcer needs; and Barlow acted willfully or recklessly through understaffing and other conduct. (TAC paras. 5, 7, 25, 34(e), 34(g), 62, 64, 71; Barlow Memorandum at pp. 10-13.)

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Barlow's defense experts characterized the wound as a stable or improving Stage III injury based primarily on surface-area reduction. (Barlow UMF Nos. 10-16, 32-42; Leo Decl. paras. 15-18, 22-26; Higelin Decl. paras. 17- 25.)

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Dr. Doo's same-date Stage IV records, the more-than-tenfold

depth increase, and Katona's progression opinion contradict the factual premise that the wound remained a stable, improving Stage III injury. (Collier Depo. 61:1-10, 62:8-63:3, 73:21-74:4; Katona Decl. paras. 9-13; Prior Order at pp. 6-7.)

The Court finds that Plaintiff has sufficiently raised a triable issue of material fact as to whether the acts or omissions of Barlow caused Decedent physical injury, pain, loss of enjoyment of life and emotional distress and anguish. Plaintiff has evidenced that triable issues of material fact exist as to whether Barlow was willfully negligent and/or reckless given the discrepancy in the characterization of Decedent's sacral wound.

Accordingly, Barlow's Motion for Summary Adjudication is DENIED.